Syncope

UP-16 | Universal Patient Care

Quick flow

01 Primary Assessment [EMT / AEMT / Paramedic]

Airway • breathing • circulation • injury • hemorrhage • pregnancy • persistent instability

02 Recovered to Baseline? [EMT / AEMT / Paramedic]

Persistent altered mental status or focal deficit requires the appropriate emergency pathway

03 Glucose + Cardiac Evaluation [EMT / AEMT / Paramedic]

Glucose on every patient • 12-lead ECG • continuous cardiac monitoring

04 High-Risk Features? [EMT / AEMT / Paramedic]

Exertional or supine • abrupt • cardiopulmonary symptoms • abnormal ECG • bleeding • abnormal vitals

05 Treat Identified Cause [EMT / AEMT / Paramedic]

Hypoglycemia • dysrhythmia • hemorrhage • shock • opioid respiratory depression • trauma

06 Transport + Reassess [EMT / AEMT / Paramedic]

Repeat vitals and ECG when indicated • monitor for recurrent symptoms • early notification

Overview

- Syncope is abrupt, transient loss of consciousness and postural tone caused by temporary global cerebral hypoperfusion, followed by rapid spontaneous recovery.
- Near-syncope may carry similar risk and receives the same initial evaluation.
- Do not label persistent altered mental status, seizure, hypoglycemia, intoxication, stroke, or traumatic loss of consciousness as uncomplicated syncope.

Provider Actions

Identify life threats and high-risk cardiac or systemic causes before considering benign syncope.

EMT

- Assess airway, breathing, circulation, mental status, injury from the fall, possible hemorrhage, pregnancy, and persistent instability.
- Obtain glucose, SpO_2, two complete vital-sign sets when feasible, and a focused neurologic examination.
- Acquire and transmit a 12-lead ECG and apply continuous cardiac monitoring for every syncope or near-syncope patient.
- Give oxygen only for hypoxemia or respiratory distress and initiate rapid transport when any high-risk feature is present.

AEMT

- Perform all EMT care. Establish IV access when hypotension, poor perfusion, clear volume depletion, recurrent symptoms, or another high-risk finding is present.
- Administer LR or normal saline only for hypotension, poor perfusion, or clear volume depletion, using the approved adult or pediatric bolus.
- Monitor the rhythm continuously, repeat complete vital signs after intervention, and prepare for cause-specific treatment.

Paramedic

- Perform all prior care. Interpret the initial 12-lead ECG and repeat it for recurrent symptoms, rhythm change, chest discomfort, or a concerning initial tracing.

- Treat bradycardia, tachycardia, ischemia, shock, hypoglycemia, respiratory failure, or another identified cause under the applicable Claiborne protocol.
- Evaluate persistent altered mental status, focal neurologic findings, suspected seizure, bleeding, ectopic pregnancy, pulmonary embolism, aortic emergency, and toxicologic causes.

High-Risk Syncope

Any high-risk feature requires continuous monitoring and prompt emergency-department evaluation.

EMT

- Syncope during exertion or while supine, or abrupt syncope without warning.
- Chest pain, dyspnea, palpitations, persistent headache, or recurrent symptoms.
- Persistent hypotension, hypoxia, bradycardia, tachycardia, abnormal perfusion, or failure to return completely to baseline.
- Known coronary disease, heart failure, cardiomyopathy, significant valvular disease, pacemaker, or implanted cardioverter-defibrillator.
- Family history of sudden unexplained death before age 50.
- GI bleeding, vaginal bleeding, abdominal or back pain, pregnancy, anticoagulant use, significant trauma, or focal neurologic deficit.

AEMT

- Establish vascular access when clinically indicated and transport without delaying for orthostatic vital signs or nonessential procedures.

Paramedic

- Treat an abnormal rhythm or identified life threat under the applicable protocol and provide early receiving-facility notification.
- Concerning ECG findings include acute ischemia, significant bradycardia or tachycardia, high-grade AV block, ventricular dysrhythmia, prolonged QT, pre-excitation, Brugada pattern, or other new conduction abnormality.

Orthostatic Vital Signs

Orthostatic testing is optional and must not provoke injury or delay transport.

EMT

- Consider only when the patient is stable, completely recovered, able to stand safely, and has no injury, pregnancy-related concern, or high-risk feature.
- A positive test is recurrent symptoms or a fall in SBP of at least 20 mm Hg or DBP of at least 10 mm Hg after standing.
- Stop immediately for dizziness, weakness, recurrent near-syncope, syncope, chest pain, dyspnea, or instability.
- Do not use normal orthostatic vital signs to exclude a serious cause and do not delay transport to obtain them.

AEMT

- When testing is appropriate, ensure monitoring and immediate assistance are available before standing the patient.

Paramedic

- Interpret orthostatic findings in the context of the full history, medication list, examination, ECG, glucose, and volume status.

Fluid Support

Fluids are for hypotension, poor perfusion, or clear volume depletion - not routine syncope treatment.

EMT

- Position the patient safely, prevent recurrent falls, and reassess perfusion while vascular access is prepared when indicated.

AEMT

- Adult: LR or normal saline 500 mL IV/IO, then reassess.
- Heart failure, renal failure, liver failure, pulmonary edema, or volume-overload risk: use 250 mL increments.
- Pediatric patient younger than 16 years: LR or normal saline 10-20 mL/kg IV/IO, then reassess.
- Stop or reduce fluid for pulmonary edema, worsening oxygenation, or resolved hypotension and poor perfusion.

Paramedic

- If hypotension or poor perfusion persists, reassess for hemorrhagic, cardiogenic, distributive, obstructive, or toxicologic shock and transition to the appropriate shock pathway.

Indications

- Syncope with transient loss of consciousness and postural tone followed by rapid spontaneous recovery.
- Near-syncope with transient lightheadedness, weakness, or impending loss of consciousness without complete loss of consciousness.
- Unexplained collapse with apparent spontaneous recovery when syncope remains possible.

Assessment | Event History

- Determine body position and activity at onset, prodrome, precipitating trigger, abruptness, duration, witnessed movements, color change, breathing, injuries, and time to complete recovery.
- Ask about exertion, standing, heat exposure, pain, emotional stress, urination, defecation, coughing, swallowing, dehydration, vomiting, diarrhea, and recent illness.
- Ask about chest pain, dyspnea, palpitations, headache, neck pain, abdominal or back pain, GI or vaginal bleeding, pregnancy possibility, and neurologic symptoms.
- Obtain cardiac, neurologic, seizure, bleeding, and thromboembolic history; family history of sudden death; and all medications, including antihypertensives, diuretics, QT-prolonging medications, and anticoagulants.

Assessment | Examination

- Assess mental status and confirm complete return to neurologic baseline. Examine pupils, speech, gaze, facial symmetry, strength, sensation, and coordination when appropriate.
- Evaluate pulse rate and regularity, blood pressure, heart and lung findings, perfusion, hydration, abdominal or back tenderness, evidence of bleeding, and injury from the fall.
- Obtain glucose on every patient. Obtain temperature when infection or environmental illness is suspected.
- Evaluate pregnancy and postpartum status when applicable. Abdominal, pelvic, or back pain with syncope in a patient who may be pregnant requires urgent ectopic-pregnancy evaluation.

Assessment | Cardiac Monitoring

- Acquire and transmit a 12-lead ECG and use continuous cardiac monitoring for every syncope or near-syncope patient.
- Repeat the ECG for recurrent symptoms, rhythm change, chest discomfort, or a concerning initial tracing.
- A single normal ECG or brief period of normal monitoring does not exclude intermittent dysrhythmia or another cardiac cause.

- Document monitor rhythm, ECG interpretation, recurrent symptoms, and any rhythm associated with those symptoms.

Treatment steps

- Support airway and ventilation and provide oxygen only for hypoxemia or respiratory distress.
- Check glucose immediately and treat hypoglycemia under UP-4.
- Acquire and transmit a 12-lead ECG and maintain continuous cardiac monitoring.
- Assess and treat injury, hemorrhage, pregnancy-related emergency, dysrhythmia, ACS, pulmonary embolism, aortic emergency, seizure, stroke, sepsis, shock, or toxicologic cause under the applicable protocol.
- Administer naloxone only when opioid exposure and clinically significant respiratory depression are suspected; do not use naloxone empirically for isolated syncope.
- Give LR or normal saline only for hypotension, poor perfusion, or clear volume depletion: adult 500 mL; volume-overload risk 250 mL; pediatric 10-20 mL/kg. Reassess after every bolus.
- Do not perform routine orthostatic vital signs. When they are safe and clinically useful, stop immediately for recurrent symptoms or instability.
- Transport every high-risk, first unexplained, recurrent, injured, pregnant, anticoagulated, abnormal-vital-sign, abnormal-glucose, or abnormal-ECG patient with continuous reassessment.

Warnings / precautions

- Transport is strongly indicated for the listed high-risk features; those criteria alone do not establish incapacity or authority for involuntary transport. If a legally authorized decision-maker refuses, assess and document decision-specific capacity, explain material risks and alternatives, contact Medical Control for high-risk refusal, and follow the approved refusal/safeguarding procedure. Use an authorized emergency or protective-custody pathway when its legal conditions apply. A patient aged 16 or 17 remains a minor for consent purposes unless a legally recognized exception applies; the clinical age-16 dosing boundary does not establish legal adulthood.
- Do not assume benign vasovagal syncope until cardiac, hemorrhagic, obstetric, neurologic, metabolic, toxicologic, and traumatic causes have been considered.
- A normal ECG does not exclude intermittent dysrhythmia or cardiac syncope.
- Do not stand an unstable, injured, pregnant, anticoagulated, or high-risk patient for orthostatic vital signs.
- Do not give routine oxygen, IV fluid, aspirin, or naloxone solely because syncope occurred.
- Syncope during exertion or while supine is high risk and requires transport for cardiac evaluation.
- Persistent altered mental status or focal neurologic deficit is not uncomplicated syncope.

Clinical pearls

- Near-syncope may carry risk similar to complete syncope and warrants the same initial evaluation.
- A detailed event history, examination, glucose, ECG, and risk assessment are more useful than routine indiscriminate testing.
- Brief myoclonic movements can occur during cerebral hypoperfusion. Persistent postictal confusion, prolonged convulsions, or lateral tongue injury increases concern for seizure.
- Isolated syncope without focal neurologic findings is not usually a stroke presentation; persistent focal findings require stroke evaluation.
- Syncope associated with dyspnea, unexplained hypoxemia, pleuritic pain, tachycardia, or thromboembolic risk requires consideration of pulmonary embolism.
- Abdominal or back pain, diminished pulses, bleeding, hypotension, or pregnancy may identify life-threatening occult hemorrhage or an aortic emergency.

Special populations | Pediatrics and athletes

- Pediatric patients are younger than 16 years and medication or fluid doses use actual or length-based weight.
- Exertional or supine syncope, chest pain, palpitations, abnormal ECG, congenital heart disease, or family history of sudden death is high risk.
- Do not permit return to sports or exertion. Transport for cardiac evaluation.

Special populations | Pregnancy and bleeding risk

- Syncope with abdominal, pelvic, or back pain or vaginal bleeding in a patient who may be pregnant requires urgent evaluation for ectopic pregnancy or other hemorrhage.
- Anticoagulant use increases concern for occult bleeding and intracranial injury after a fall.
- Do not delay transport for orthostatic vital signs when bleeding, pregnancy complication, or significant trauma is possible.

Special populations | Refusal / Non-Transport

- Transport is strongly indicated for any high-risk feature, first unexplained or recurrent event, abnormal ECG, glucose or vital signs, injury, pregnancy, anticoagulant use, persistent symptoms, or incomplete return to baseline.
- A capable adult with a classic vasovagal trigger and prodrome, complete recovery, normal examination, glucose, vital signs, and ECG, and no injury or high-risk feature may refuse only after Medical Control consultation.
- Complete the Refusal / Non-Transport protocol and document capacity, event history, examination, ECG and glucose results, risks explained, Medical Control consultation, responsible adult supervision, and return precautions.

Source and references

Tennessee EMS BLS/ALS State Protocol Guidelines 2024-2025 - SOP 319 Syncope. | ACC / AHA / HRS - Guideline for the Evaluation and Management of Patients With Syncope, 2017. | Claiborne EMS UP-4 Diabetic / Glucose Emergencies and applicable cardiac, shock, trauma, obstetric, stroke, seizure, and toxicology protocols. | North Carolina College of Emergency Physicians UP-16 Syncope source protocol retained for historical comparison.

Revision: October 31, 2026 | Last verified: October 31, 2026